

Tesamorelin

GHRH Analogue | FDA-Approved (Egrifta) | 5mg & 10mg x 10 Vials

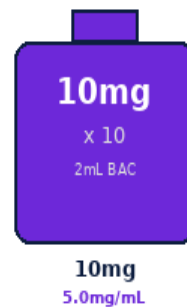
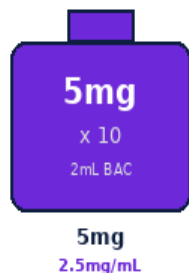
Clinically Proven Visceral Fat Reduction | Once Daily Fasted

Purity: 99.8% | Endotoxin-Free | Research Use Only

Tesamorelin is a stabilised GHRH analogue and the only FDA-approved GHRH peptide (as Egrifta for HIV-associated lipodystrophy). It stimulates physiological GH release and is the most clinically validated GHRH for visceral fat reduction. Must be injected fasted once daily.

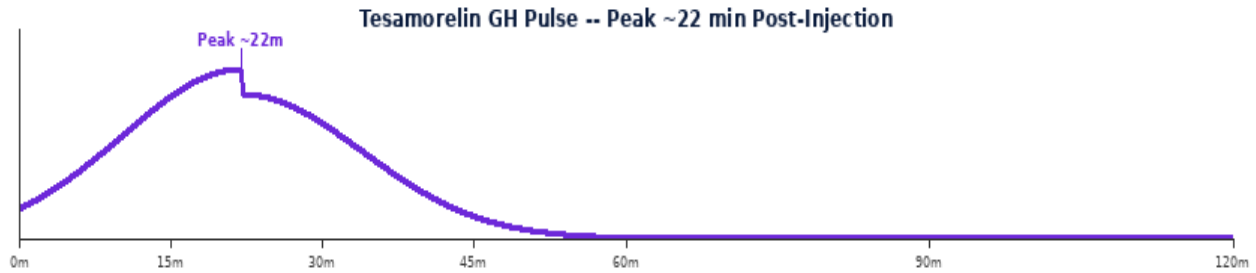
FASTED REQUIRED: Inject at least 1-2 hours after last meal. Insulin blunts GH release. Wait 30 min before eating post-injection.

Tesamorelin Vial Comparison



Parameter	Specification
Half-Life	~26-38 minutes -- stimulates natural GH pulse
Vial Sizes	5mg x 10 10mg x 10 Purity 99.8%
Reconstitution	2 mL BAC Water per vial: 5mg->2.5mg/mL 10mg->5mg/mL
Standard Dose	1,000-2,000 mcg (1-2 mg) once daily SubQ
Frequency	Once daily -- always fasted, morning or bedtime
Key Benefit	Clinically proven visceral fat reduction; only FDA-approved GHRH
Cycle	12-26 weeks; can run longer with monitoring
Storage	2-8 deg C unconstituted 2 mL BAC Water use within 28 days

MECHANISM OF ACTION & RESEARCH BENEFITS



Pathway	Mechanism	Benefit
GHRHR	Binds GHRH receptors on pituitary somatotrophs; activates Gs/cAMP/PKA; drives GH synthesis and release	Physiological GH pulse once daily
Lipolysis	GH-mediated HSL activation; preferential reduction of visceral/abdominal adipose tissue; clinically measured by CT scan in trials	Proven visceral fat reduction; trunk fat loss
IGF-1	Daily GH pulses progressively raise IGF-1; lean mass preservation; recovery enhancement over cycle	Body composition improvement; anti-aging

Benefit	Context
FDA Validation	Tesamorelin (Egrifta) is FDA-approved and has the strongest clinical evidence base of any GHRH peptide for fat reduction.
Visceral Fat	CT-scan confirmed reduction in visceral adipose tissue in clinical trials -- superior evidence to other GHRH analogues.
GHS Synergy	Stacking with Ipamorelin amplifies GH pulse 3-5x. Tesamorelin + Ipamorelin is a premium body recomposition stack.

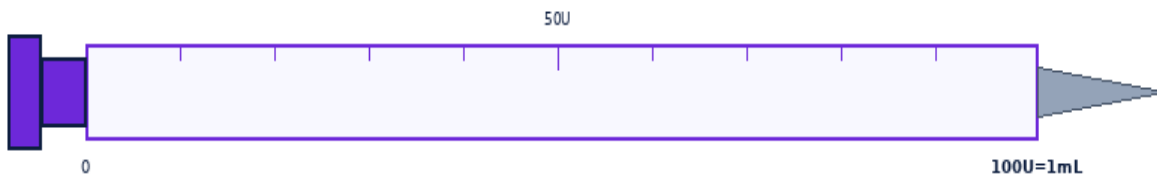
RECONSTITUTION & DOSE CALCULATION

All vials use 1 mL BAC Water. Room temperature. Swirl gently -- never shake.



1 mL BAC Water per vial. Roll gently -- NEVER shake.

Step	Action	Detail
1	Gather	Vial, BAC Water, insulin syringe (29-31G), swabs, sharps.
2	Clean stoppers	Wipe both stoppers. Air-dry 10-15 sec.
3	Draw 1mL BAC	Exactly 1 mL. Expel air bubbles.
4	Inject into vial	Aim at GLASS WALL. Slowly. Never onto powder.
5	Swirl & label	Roll 20-30 sec. Clear within 60 sec. Never shake. Label date. 2-8 deg C. Use within 28 days.



1mL Insulin Syringe | 29-31G | SubQ

Formula: Vol(mL)=Dose(mcg)/Conc(mcg/mL) | **Units:** Vol x 100

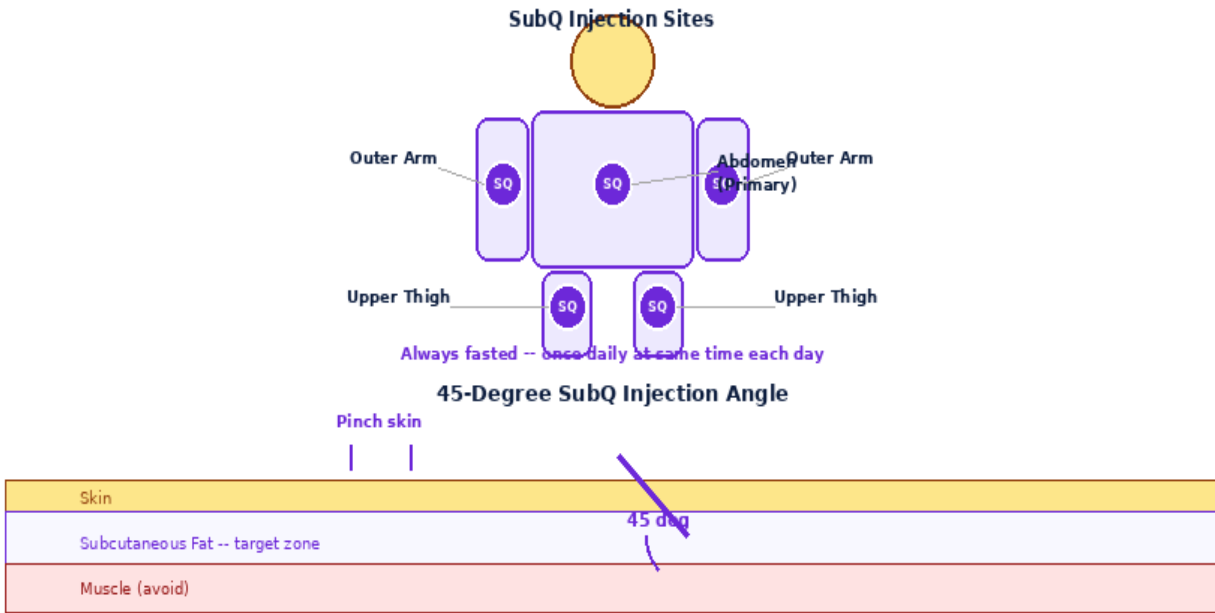
5mg x 10 -- 2mL BAC -> 2.5mg/mL (2,500mcg/mL)

Dose(mcg)	500mcg	750mcg	1000mcg	1500mcg	2000mcg
Vol(mL)	0.2000	0.3000	0.4000	0.6000	0.8000
Units	20.0U	30.0U	40.0U	60.0U	80.0U
Doses/Vial	5	3	2	1	1

10mg x 10 -- 2mL BAC -> 5.0mg/mL (5,000mcg/mL)

Dose(mcg)	500mcg	750mcg	1000mcg	1500mcg	2000mcg
Vol(mL)	0.1000	0.1500	0.2000	0.3000	0.4000
Units	10.0U	15.0U	20.0U	30.0U	40.0U
Doses/Vial	10	6	5	3	2

INJECTION TECHNIQUE, PROTOCOLS & GOLDEN RULES



8-Point Rotation Map -- Rotate Every Injection



Step	Action	Detail
1	Confirm fasted	1-2 hr after last meal. Mandatory.
2	Draw dose	Draw 0.4-0.8mL (1,000-2,000 mcg) per dose table.
3	Inject SubQ	Abdomen preferred. Pinch 2-3cm. 45 deg. Inject slowly.
4	Wait & rotate	Wait 30 min before eating. Rotate 8-point map.

Protocol	Dose	Frequency	Duration	Notes
Standard	1,000 mcg	1x daily fasted	12-26 wk	Morning or bedtime fasted.
High	2,000 mcg	1x daily fasted	12-26 wk	Max FDA dose. Monitor IGF-1.
+IPA Stack	1,000 mcg +200mcg Ipa	Both daily fasted	12 wk	Inject simultaneously. Best recomp protocol.

Side Effect	Likelihood	Management
Injection site reaction	Common	Rotate sites. Use fresh needle.
Water retention	Occasional	GH-mediated. Reduce dose.
Glucose changes	Occasional	Monitor fasting glucose at high doses.
Do not use in cancer history	Contraindicated	GH/IGF-1 elevation risk.

Contraindications: Cancer history, pregnancy, under 18.

1. Inject fasted (1-2hr after last meal). Insulin blunts GH release.
2. 2 mL BAC Water per vial: 5mg->2.5mg/mL 10mg->5mg/mL.
3. Once daily injection -- morning fasted or pre-bed are optimal.
4. Stack with Ipamorelin for amplified GH pulse and superior body composition results.
5. Do not use in any cancer history -- GH/IGF-1 stimulation is contraindicated.
6. Rotate injection sites. Store 2-8 deg C. Use within 28 days.
7. Cycle 12-26 weeks. Monitor IGF-1 levels periodically on long cycles.

DISCLAIMER: research use only. Not for human therapeutic use. Consult a qualified professional. assumes no liability.
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